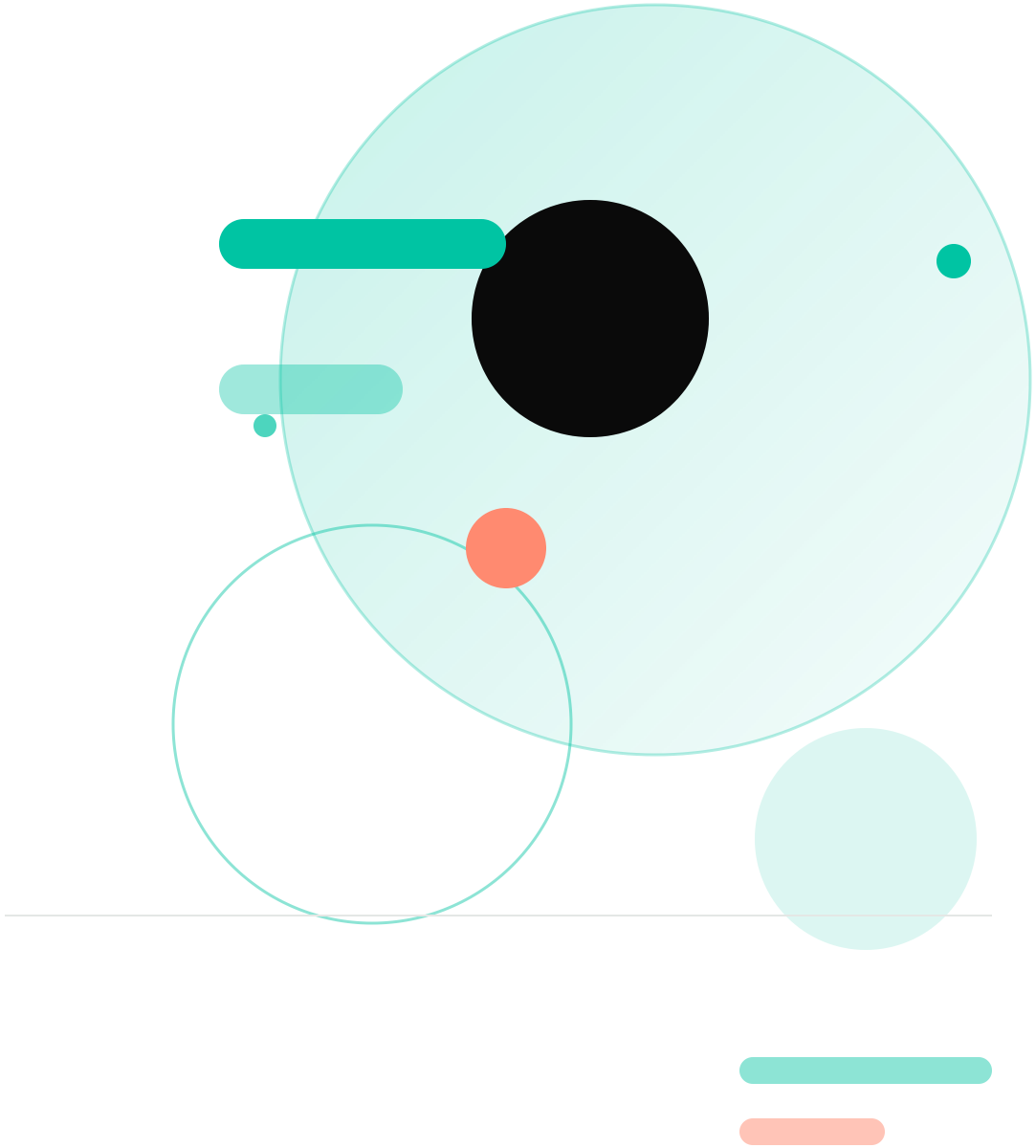
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REVISED ENGAGEMENT PROPOSAL · REF MZ-ENG-DAMC-2026-R2

# The plan for Dennis Ashley

What we found, what we bring, what it earns, and what we will ask for next.

Prepared for Dr. Chima Oti, Managing Director, and the Board · 25 September 2026

Private & Confidential · Mezo, a Consult For Africa vehicle

WHAT WE FOUND ON THE VISIT

# The asset is a long way ahead of the trading.

### The building is not the problem

The address, the fit-out, the endoscopy suite and the name carry more weight in this market than the current level of activity reflects. The gap is demand and operating rhythm, not bricks, equipment or clinical quality.

### No demand engine behind it

A premium clinic at this address should not be waiting to be found. There is a presence on social media. There is no marketing programme, referral pipeline, published price card or booking route a stranger can use.

### The capacity is already paid for

Two consulting rooms, a treatment room and an endoscopy suite with recovery, sitting idle for most of the week. Every session added earns against rent, equipment and staff the clinic is already carrying.

None of this is a criticism of the board. It is what happens in any practice below its capacity, where nobody has been given the brief to close the gap. **That brief is the engagement.**

WHAT WE BRING

# Traction, not intentions

| Vehicle | What it is today | What it runs at Oniru |
|---|---|---|
| **Paras Orthocare** | An operating orthopaedic and arthroscopy practice | Ortho and arthroscopy consultation lists |
| **Osteon Clinics** | An orthopaedic centre with its own theatres and surgical lists | Island consultation and post-operative clinics; surgery at Osteon |
| **Nelia** | Women's health and regenerative medicine practice | Women's health clinics and treatment-room procedures |
| **Cooked Indoors**<br>Balance Clinic · Sito Metabolic | Two clinics taking bookings online today | Weight management and metabolic programmes, and the scopes they generate |
| **GetSlim** | Bariatric and general surgery with its own endoscopy lists | Surgical clinics, and gastroscopy and colonoscopy into your suite |
| **Haven Paediatrics** | Established Ikeja practice seeking an Island extension | Paediatric sessions, and cover for employees' children |

Not a recruitment plan, a hiring budget or a marketing hypothesis. **Demand that already exists, redirected to an address with the room to take it.** Urology is next.

WHAT THAT DOES TO THE WEEK

# Two rooms and a suite, working to nine rhythms instead of one

| | Consulting rooms | Endoscopy suite | Treatment room |
|---|---|---|---|
| **Mon** | Dennis Ashley general and wellness · Nelia women's health | Corporate screening workup | Infusions and minor procedures |
| **Tue** | Osteon orthopaedics · Balance Clinic weight management | **Endoscopy list** | Injections and regenerative |
| **Wed** | Dennis Ashley · urology | Corporate and executive health | Minor procedures |
| **Thu** | Paras Orthocare arthroscopy · GetSlim bariatric and general surgery | **Endoscopy list** | Pre-assessment and post-op |
| **Fri** | Dennis Ashley · Sito metabolic and diabetes | Flexible list | Infusions and dressings |
| **Sat** | Haven Paediatrics · executive health | **Endoscopy list** | Screening and phlebotomy |

Bariatric assessment requires endoscopy by protocol. **The suite fills by clinical necessity, not persuasion**, against equipment already paid for.

THE BUILD-UP, BOTTOM UP

# Built from rooms, sessions and lists. Not from percentages.

**N30.9m**
Month 12 run rate, a month, against roughly N2.5m today

**N201m**
Year one revenue, against N30m if nothing changes

**N14.5m**
Month 12 contribution, a month, before fixed costs

**84%**
Of year one is revenue that does not exist today

This is a **jumpstart, not an optimisation**. We hold today's trading flat for twelve months and claim credit for none of it. Everything above that line is ours to produce.

WHAT THE PLAN ACTUALLY ASKS FOR

# The naira are large because Lagos specialist prices are large. The activity is small.

## 9 a day

Extra patients on a working day by month twelve, across two rooms and six visiting practices. Roughly three per room.

## 4 a week

Endoscopy cases by month twelve, against a suite that could take twenty-four. Bariatric workup alone should carry most of it.

## 42%

Of sellable room sessions in use at month twelve. Half the building is still idle at the end of year one.

Specialist care, not primary care, and an **80:20 cash to payer mix**. Less credit business means the clinic collects at the point of care rather than ninety days later, and funds its growth out of cash instead of receivables.

WHO DOES WHAT, AND WHO FUNDS WHAT

# We bring the capability. The clinic carries its own costs, as it does today.

### Mezo provides, at our cost

The management team and the operating discipline. The platform, with billing, booking, claims tracking and live dashboards. Financial management and collections. Procurement at network volumes, including vendor-managed inventory. The bench and the demand engine.

### Dennis Ashley funds, as today

**The existing staff stay on your payroll, unchanged: we manage them, you employ them.** Rent, utilities, licences and indemnity. Consumables, pharmacy stock and laboratory outsourcing. And one new line: an agreed monthly marketing budget, N750,000 as a working figure, for content and paid media.

A presence on social media is not a demand engine. **The marketing budget is the one thing we are asking the board to fund, and it is the cheapest line in this plan.**

THE TERMS, PLAINLY

# Yours to own. Ours to run. Paid on cash that arrives.

- **Dennis Ashley remains 100% yours:** ownership, brand, premises and clinical governance unchanged.
- **Management fee: 7.5% of monthly collected revenue,** invoiced in arrears. Collected, not billed.
- **Performance incentive** earned on growth milestones measured against an agreed baseline.
- **The clinic bills the patient for everything.** A bench practitioner is paid a professional-fee share of their own work; the diagnostics, pharmacy, treatment room and suite revenue stay wholly with the clinic.
- **Nobody is paid for a referral,** which keeps the arrangement inside MDCN ethics and your own governance.
- **Either side can exit on agreed notice.** The standards, the data and the improvements stay with the clinic.

WHAT WE NEED FROM YOU

# What we need back from you.

Everything for the audit sits on one private page: the nine items that matter most, somewhere to upload them straight from the page, and four short surveys for staff, patients, referring colleagues and the board. The surveys need no paperwork at all, so they are the easiest place to start.

> consultforafrica.com/DennisAshleyProject

Confidential to us, used only for the audit, returned or destroyed if we do not proceed. No patient-identifiable data is requested. The mutual NDA is yours whenever you want it, and we would rather it did not become the reason nothing moves.

THE ONE DECISION

# Give us a week in October.

The audit is at our cost and without obligation, and the plan in these slides does not depend on it. What the audit does is fix the baseline we are measured against, so the targets in the Management Agreement are written against your numbers rather than our estimates.

1

THIS WEEK

### A week in October

You and the Administrator name it. We build the on-site days around your clinic schedule, and we come for an endoscopy list if one is running.

2

WEEKS ONE TO THREE

### The audit runs

Alongside normal operations, without disrupting them. Uploads and surveys can start today.

3

WEEK FOUR

### Findings, then signature

Findings to the board, then the Management Agreement drafted against the baseline.

Dennis Ashley is the right address, with the right brand, in the right market. What it has been missing is volume, and **volume is what we are bringing through the door.**